



The Intersection Between Professionalism Assessment and Competency-Based Medical Education

Greg Ogrinc, MD, MS, SVP Certification Standards and Programs, ABMS

Lisa Taitzman, MD, MPH, ABOS Board of Directors

Ariel Winn, MD, Boston Children's Hospital

Disclosures

- Dr. Ogrinc is a paid employee of the American Board of Medical Specialties
- Dr. Ogrinc receives royalties from Joint Commission Resources
 - *Fundamentals of Health Care Improvement*
 - *Practical Measurement for Health Care Improvement*
- Dr. Winn is funded by the American Board of Pediatrics

Learning Objectives

- Describe the approach taken by ABMS to create collaboration across member boards in CBME and promoting professionalism
- Discuss how the principles of CBME can be used to facilitate assessment of professional behaviors in residency education programs to inform professionalism attestations for certification
- Explain how direct member board assessment of professionalism in residency education can be achieved through implementation of CBME

Outline of this Presentation

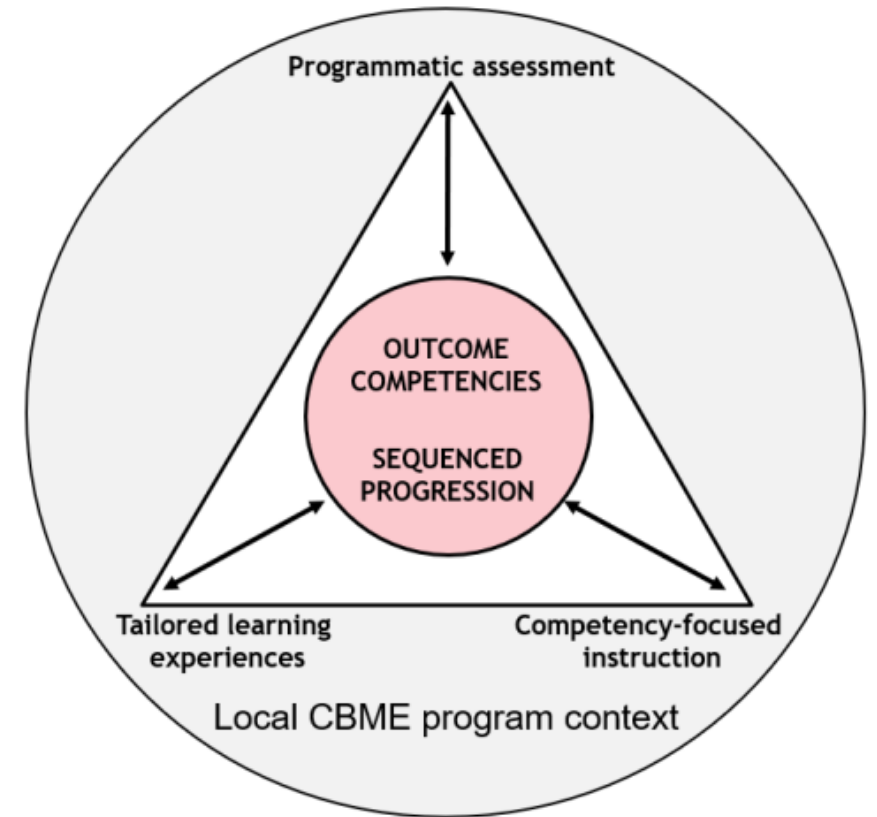
- ABMS competency-based medical education (CBME) and promoting professionalism (15 min)
 - CBME Learning Community with ACGME
 - Promoting Professionalism Action Collaborative
- ABOS knowledge, skills, and behaviors platform (15 min)
- ABP assessment of professional behaviors (15 min)
- Questions, Comments, Discussion (10-15 min)

Overview of CBME

- Competency-based, time-variable education is superior to time-based, competency-variable education
- Implementation has been sloooooow
- COVID-19 pandemic
 - Need for competency-based approaches (modified time and volume-based metrics)
 - Competency-based decisions possible
- Commitment of ABMS and ACGME to collaborate re: implementation of CBME

Competency-based medical education (CBME)

- CBME is how outcomes-based education gets operationalized and implemented.
- Defined in 2010 by International Medical Education Collaborative as...
*...an approach to preparing physicians for practice that is fundamentally oriented to graduate outcome abilities and organized around competencies derived from an analysis of **societal and patient needs**.*
- The Core Components Framework helps to define the key aspects of CBME



Key Philosophical Principles

- Grounded in a “reform/growth” mindset
- Focused on promoting learner *growth and development* through frequent *formative* assessment (assessment *for* learning > assessment *of* learning)
- Rich in feedback/coaching individualized to the learner
- Provides rich and diverse learning experiences, steeped in clinical practice where learners can stay *as long as required*

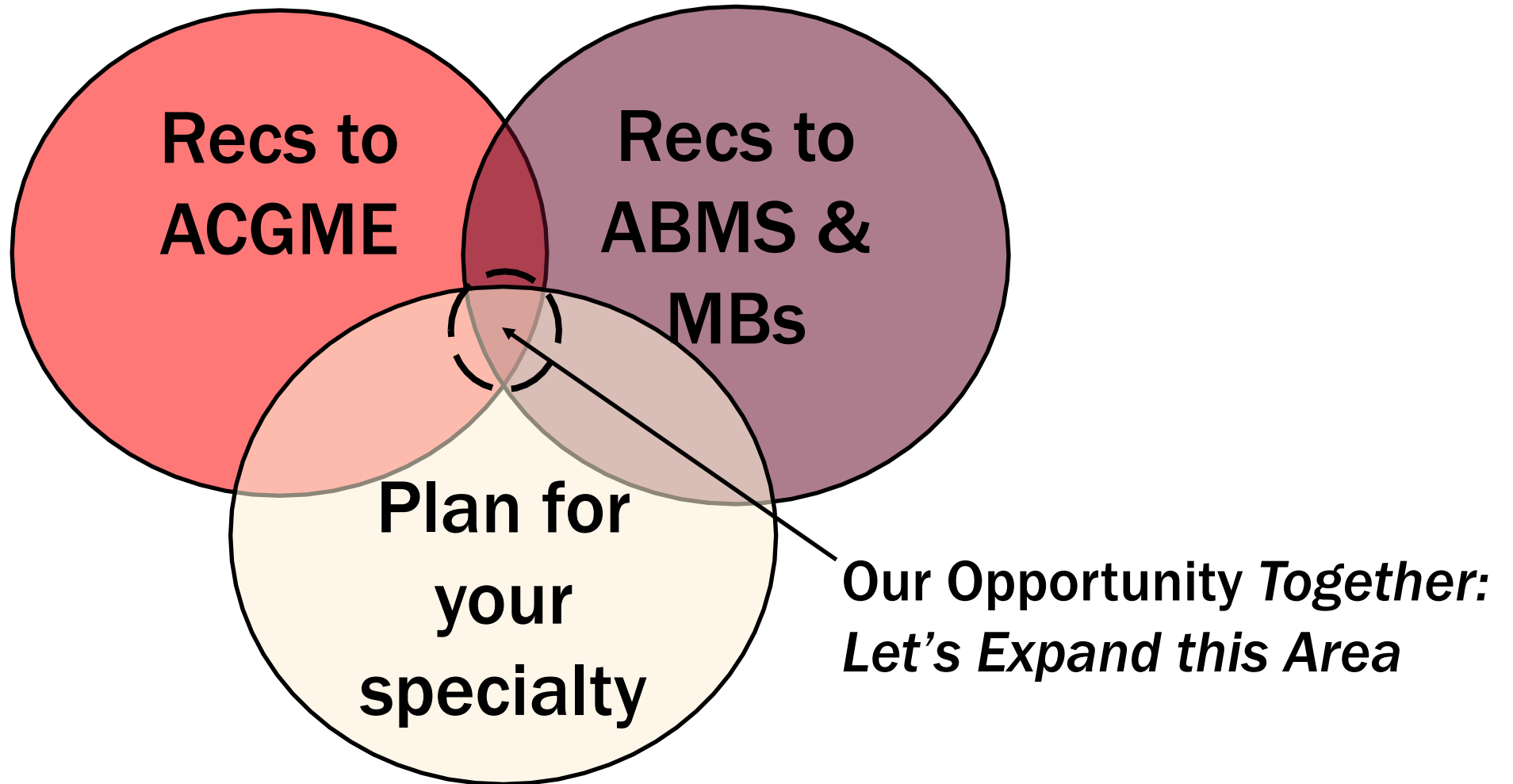
CBME LC Goals, Objectives, Outcomes

Accelerate the development of and transition to competency-based medical education (CBME) in graduate medical education (GME) in the U.S.

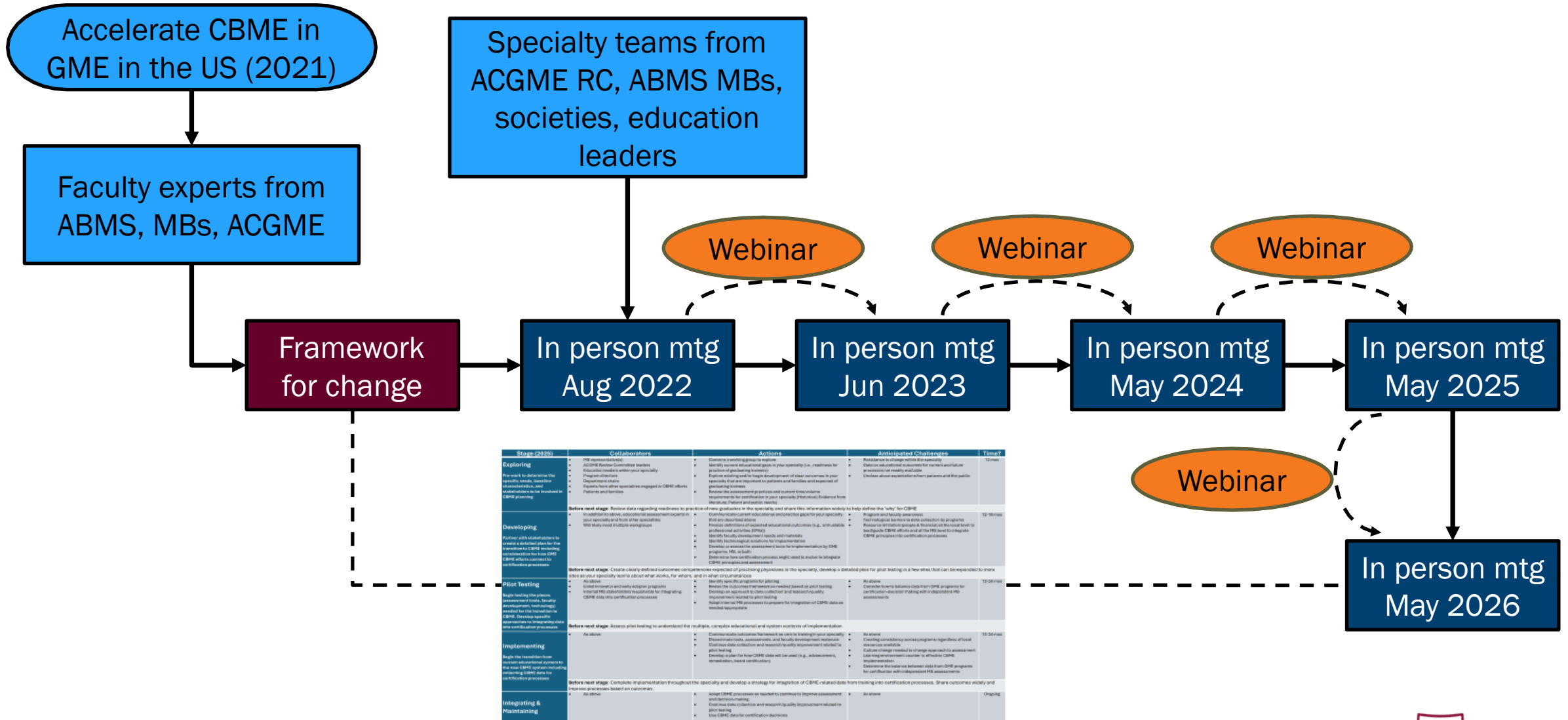
Objectives

1. Create a shared understanding of effective practices for implementing CBME across different contexts.
2. Identify the policy, financial, and administrative facilitators that have empowered spread and innovation in CBME.
3. Identify the policy, financial, and administrative barriers that inhibit the growth of CBME.
4. Recommend changes in ACGME and ABMS policies and procedures that promote innovation and reduce or eliminate barriers to CBME.
5. Working within and across specialties, create an action plan to support innovations and the widespread implementation of CBME.

CBME LC Outcomes



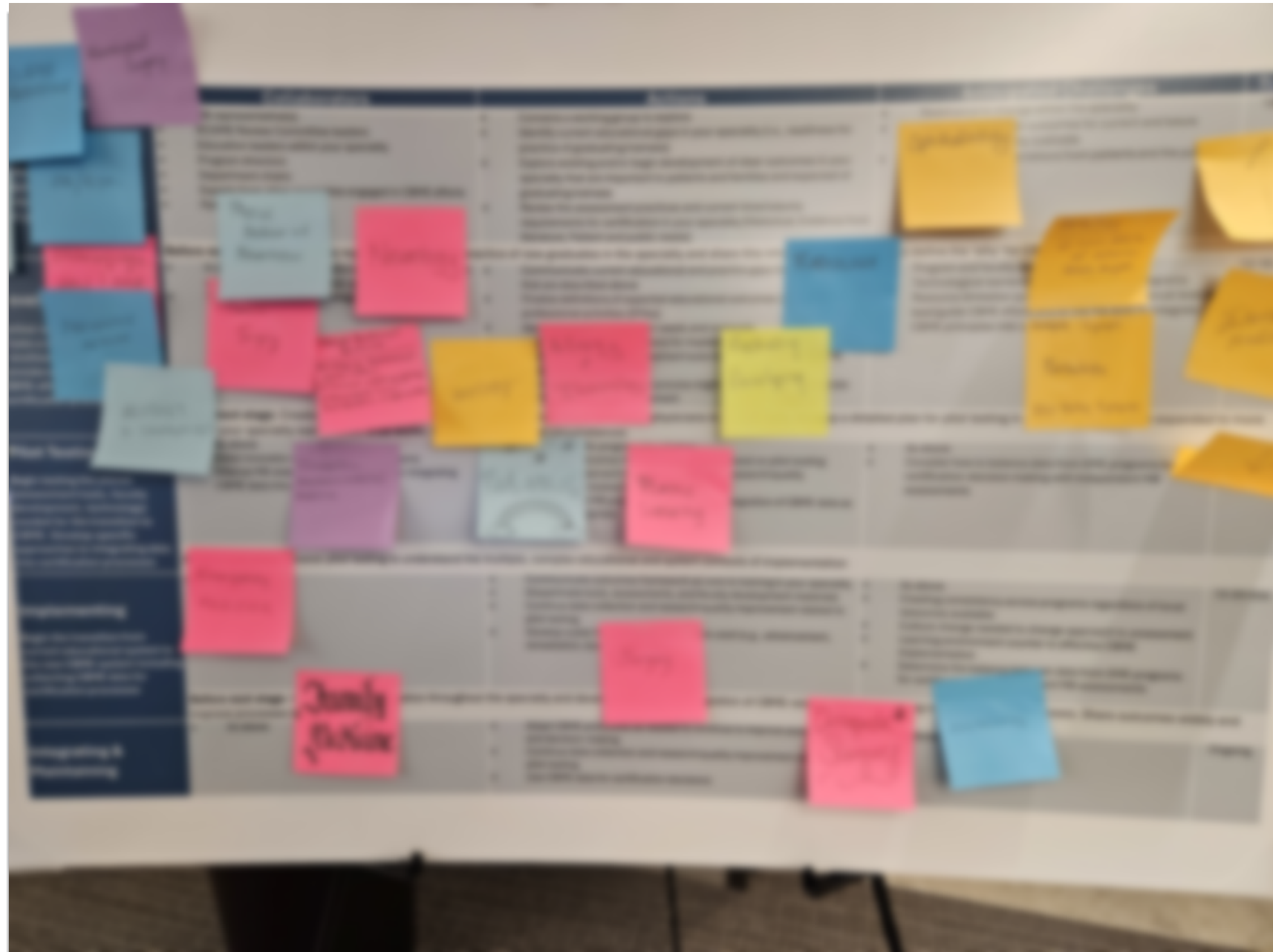
ABMS ACGME CBME Learning Community



Stages of Competency Based Medical Education (CBME) Development within a Specialty

Stage	Collaborators	Actions	Anticipated Challenges	Time?
Exploring Pre-work to determine the specific needs, baseline characteristics, and stakeholders to be involved in CBME planning	• MB representative(s) • ACGME Review Committee leaders • Education leaders within your specialty • Program directors • Department chairs • Experts from other specialties engaged in CBME efforts • Patients and families	• Convene a working group to explore • Identify current educational gaps in your specialty (i.e., readiness for practice of graduating trainees) • Explore existing and/or begin development of clear outcomes in your specialty that are important to patients and families and expected of graduating trainees • Review the assessment practices and current time/volume requirements for certification in your specialty (Historical; Evidence from literature; Patient and public needs)	• Resistance to change within the specialty • Data on educational outcomes for current and future processes not readily available • Unclear about expectations from patients and the public	12 mos
	Before next stage: Review data regarding readiness to practice of new graduates in the specialty and share this information widely to help define the 'why' for CBME			
Developing Partner with stakeholders to create a detailed plan for the transition to CBME including consideration for how GME CBME efforts connect to certification processes	• In addition to above, educational assessment experts in your specialty and from other specialties • Will likely need multiple workgroups	• Communicate current educational and practice gaps for your specialty that are described above • Finalize definitions of expected educational outcomes (e.g., entrustable professional activities (EPAs)) • Identify faculty development needs and materials • Identify technological solutions for implementation • Develop or assess the assessment tools for implementation by GME programs, MB, or both • Determine how certification process might need to evolve to integrate CBME principles and assessment	• Program and faculty awareness • Technological barriers to data collection by programs • Resource limitation (people & financial) at the local level to lead/guide CBME efforts and at the MB level to integrate CBME principles into certification processes	12-18 mos
	Before next stage: Create clearly defined outcomes competencies expected of practicing physicians in the specialty, develop a detailed plan for pilot testing in a few sites that can be expanded to more sites as your specialty learns about what works, for whom, and in what circumstances			
Pilot Testing Begin testing the pieces (assessment tools, faculty development, technology) needed for the transition to CBME. Develop specific approaches to integrating data into certification processes	• As above • Enlist innovator and early adopter programs • Internal MB stakeholders responsible for integrating CBME data into certification processes	• Identify specific programs for piloting • Revise the outcomes framework as needed based on pilot testing • Develop an approach to data collection and research/quality improvement related to pilot testing • Adapt internal MB processes to prepare for integration of CBME data as needed/appropriate	• As above • Consider how to balance data from GME programs for certification-decision making with independent MB assessments	12-24 mos
	Before next stage: Assess pilot testing to understand the multiple, complex educational and system contexts of implementation			
Implementing Begin the transition from current educational system to the new CBME system including collecting CBME data for certification processes	• As above	• Communicate outcomes framework as core to training in your specialty • Disseminate tools, assessments, and faculty development materials • Continue data collection and research/quality improvement related to pilot testing • Develop a plan for how CBME data will be used (e.g., advancement, remediation, board certification)	• As above • Creating consistency across programs regardless of local resources available • Culture change needed to change approach to assessment • Learning environment counter to effective CBME implementation • Determine the balance between data from GME programs for certification with independent MB assessments	12-24 mos
	Before next stage: Complete implementation throughout the specialty and develop a strategy for integration of CBME-related data from training into certification processes. Share outcomes widely and improve processes based on outcomes.			
Integrating & Maintaining	• As above	• Adapt CBME processes as needed to continue to improve assessment and decision-making • Continue data collection and research/quality improvement related to pilot testing	• As above	Ongoing

May 2025, CBME Poster with Team Self-Assessment



Stages of Competency Based Medical Education (CBME) Development within a Specialty for Graduate Medical Education (GME) and Certifying Member Boards (MB) - 2024

Stage	Collaborators	Actions	Anticipated Challenges	Time?
Exploring Otolaryngology Head and Neck Surgery Thoracic Surgery Pre-work to determine the specific needs, baseline characteristics, and stakeholders to be involved in CBME planning	- MB representative(s) - ACGME Review Committee leaders - Department chairs - Experts from other specialties engaged in CBME efforts	- Convene a working group to explore - Identify educational gaps in your specialty (i.e., readiness of residents or fellows) - Review the outcomes in your specialty that are expected of graduating trainees to patients and families and expected of graduating trainees - Review the current time and volume requirements for certification in your specialty - Evidence-based from literature	- Resistance to change within the specialty - Data on educational outcomes for current and future patients - Unclear a path from patients and the public	12 mos
Developing Partner with stakeholders to create a detailed plan for the transition to CBME including consideration for how GME CBME efforts connect to certification processes	- In addition to above, educational assessment efforts in your specialty and from other specialties - Will likely need multiple workgroups	- Finalize and disseminate outcomes for your specialty that are described above - Partner to identify faculty development needs and materials - Identify technological solutions to implementation - Partner to develop assessment tools for implementation by GME programs, MB, or both - Determine how certification process might need to evolve to integrate CBME principles into assessment	- Program and faculty awareness - Technological barriers to data collection by programs - Resource limitation (people & financial) at the local level to lead/guide CBME efforts and at the MB level to integrate CBME principles into certification processes	12-18 mos
Pilot Testing Begin testing the pieces (assessment tools, faculty development, technology) needed for the transition to CBME. Develop specific approaches to integrating data into certification processes.	- Assessment tools "Early adopter" programs - Internal MB stakeholders responsible for integrating CBME data into certification processes	- Identify specific programs for piloting - Create expectations and metrics for pilot programs - Develop an approach to data collection and research/continuous quality improvement related to pilot testing - Adapt internal MB processes to prepare data as needed/appropriate	- As above - Consider how to balance data from GME programs for certification-decision making with independent MB assessments	12-24 mos
Implementing Begin the transition from current educational system to the new CBME system including collecting CBME data for certification processes	As above	- Partner (as appropriate) to disseminate tools, assessments, and faculty development materials - Collect CBME data for MB certification decision-making	- As above - Creating consistency across programs regardless of local resources available - Culture change needed to change approach to assessment - Challenges in the learning environment counter to effective CBME implementation - Determine the balance between data from GME programs for certification with independent MB assessments	12-24 mos
Integrating & Maintaining Maintain and improve GME implementation to integrate CBME data into certification processes	As above	- Adapt processes as needed to continue to improve assessment - Use CBME data for certification decisions	As above	Ongoing

Stages of Competency Based Medical Education (CBME) Development within a Specialty for Graduate Medical Education (GME) and Certifying Member Boards (MB) - 2025

Stage (2)	Collaborators	Action	Anticipated	Time?
Exploring Pre-work to determine the specific needs and characteristics stakeholders to CBME planning	• No representative(s) • ACGME Review Committee leaders • Education leaders within your specialty • Program directors • Chairs from other specialties engaged with residents and families	• Convene a working group to explore • Identify current educational gaps in your specialty (i.e., readiness for practice of graduating trainees) • Explore existing and develop clear outcomes in your specialty that patients and families and expect • Review the assessment practices and current requirements for certification in your specialty (e.g., literature; Patient and public needs)	• Resistance to change within the specialty • Data on educational outcomes for current and future processes not readily available • Unclear about expectations from patients and the public	12 mos
	• Collect data regarding readiness to practice of new graduates in the specialty and share this information widely to help define the 'why' for CBME			
Developing Partner with stakeholders to create a detailed plan for the transition to CBME including consideration for how GME CBME efforts connect to certification processes	• In addition to the educational assessment experts in your specialty and from other specialties • Multidisciplinary workgroup	• Communicate current educational and practice gaps for your specialty that are described above • Identify faculty development needs and materials • Identify technological solutions for implementation • Assess the assessment tools for implementation by GME MB, or both • Determine how certification process might need to evolve to integrate CBME principles and assessment	• Program and faculty awareness • Technological barriers to data collection by programs • Resource limitation (people & financial) at the local level to integrate CBME principles into certification	12-18 mos
	Before next stage: Create clearly defined outcomes competencies expected of practicing physicians in the specialty, develop a detailed plan for pilot testing in a few sites that can be expanded to more sites as needed			
Pilot Testing Begin testing the pieces (assessment tools, faculty development, technology) needed for the transition to CBME. Develop specific approaches to integrating data into certification processes	• As above • Enlist innovator and early adopters • Internal MB stakeholders responsible for integrating CBME data into certification processes	• Identify specific program • Revise the outcomes framework as needed based on pilot testing • Develop an approach to data collection and research/quality improvement related to pilot testing • Adapt internal MB processes to prepare for integration of CBME data as needed/appropriate	• Consider how to balance data from GME programs for certification-decision making with independent MB assessments	12-24 mos
	Before next stage: Assess pilot testing to understand the multiple, complex educational and system contexts of implementation			
Implementing Begin the transition from current educational system to the new CBME system including collecting CBME data for certification processes	• As above	• Communicate outcomes framework as core to training in your specialty • Disseminate tools, assessments, and faculty development materials • Continue data collection and research/quality improvement related to pilot testing • Develop a plan for how CBME data will be used (e.g., advancement, remediation, board certification)	• As above • Creating consistency across programs regardless of local resources available • Culture change needed to change approach to assessment • Learning environment counter to effective CBME implementation • Determine the balance between data from GME programs for certification with independent MB assessments	12-24 mos
	Before next stage: Complete implementation throughout the specialty and develop a strategy for integration of CBME-related data from training into certification processes. Share outcomes widely and improve processes based on feedback			
Integrating & Maintaining	• As above	• Adapt CBME processes as needed to continue to improve assessment and decision-making • Continue data collection and research/quality improvement related to pilot testing • Use CBME data for certification decisions	• As above	Ongoing

Stages of Competency Based Medical Education (CBME) Development within a Specialty for Graduate Medical Education (GME) and Certifying Member Boards (MB) - 2026

Stage	Collaborators	Actions	Anticipated Challenges	Time?
Exploring Pre-work to determine the specific needs, baseline characteristics, and stakeholders to be involved in CBME planning	- MB representative(s) - ACGME Review Committee leaders - Education leaders within your specialty - Program directors - Department chairs - Other stakeholders - Patients and families	- Convene a working group to explore - Identify current educational gaps in your specialty (i.e., readiness for practice; readiness of trainees) - Explore development of clear outcomes in your specialty to patients and families and expected of graduating trainees - Review the assessment practices and current time/volume requirements for certification in your specialty (Historical; Evidence from literature; Patient and public needs)	- Resistance to change within the specialty - Data collection and future processes - Unclear about patients and the public	12 mos
Before next stage: Review data regarding readiness to practice of new graduates in the specialty and share this information widely to help define the 'why' for CBME				
Develop Partner with stakeholders to create a detailed plan for the transition to CBME including consideration for how GME CBME efforts connect to certification processes	- In addition to above, educational assessment experts in your specialty and from other specialties - Other stakeholders	- Communicate current educational and practice gaps to faculty and faculty awareness that are described above - Finalize definitions of expected educational outcomes (e.g., entrustable professional activities (EPAs)) - Identify faculty development materials - Identify technological solutions for implementation - Develop or assess the assessment tools for implementation by GME programs, MB, or both - Determine how certification process might need to evolve to integrate CBME principles and assessment	- Local barriers to data collection by programs - Resource limitation (people & financial) at the local level to lead/guide CBME efforts and at the MB level to integrate CBME principles into certification processes	12-18 mos
Before next stage: Create clearly defined outcomes expected of practicing physicians in the specialty, develop a plan for pilot testing in a few sites that can be expanded to more sites as your specialty learns about what works, for whom, and in what circumstances				
Pilot Testing Begin testing the pieces (assessment tools, faculty development, technology) needed for the transition to CBME. Develop specific approaches to integrating data into certification processes	- As above - Enlist innovator and early adopter programs - Internal MB stakeholders responsible for integrating CBME data into certification processes	- Identify specific programs for piloting - Review outcomes framework as needed - Develop data collection and research/quality improvement related to pilot testing - Adapt internal MB processes to prepare for implementation of data as needed/appropriate	- As above - Consider how to balance data from GME programs for certification-decision making with independent assessments	12-24 mos
Before next stage: Assess pilot testing to understand the multiple, complex educational and system contexts of implementation				
Implementing Begin the transition from current educational system to the new CBME system including collecting CBME data for certification processes	As above	- Communicate outcomes framework as core to training in your specialty - Disseminate tools, assessments, and faculty development materials - Continue data collection and research/quality improvement related to pilot testing - Develop a plan for how CBME data will be used (e.g., advancement, remediation, board certification)	- As above - Creating consistency across programs regardless of local resources available - Culture change needed to change approach to assessment - Learn from effective CBME implementation - Determine the balance between data from GME programs for certification with independent MB assessments	12-24 mos
Before next stage: Complete implementation throughout the specialty and develop a strategy for integration of CBME-related data from training into certification processes. Share outcomes widely and improve processes based on outcomes.				
Integrating & Maintaining	As above	- Adapt CBME processes as needed to continue to improve assessment and decision-making - Continue data collection and research/quality improvement related to pilot testing - Use CBME data for certification decisions	As above	Ongoing

Internal Medicine

Pediatrics

Orthopaedic Surgery

Stages of Competency Based Medical Education (CBME) Development within a Specialty for Graduate Medical Education (GME) and Certifying Member Boards (MB) - 2026

Stage	Collaborators	Actions	Anticipated Challenges	Time?
Exploring Pre-work to determine the specific needs, baseline characteristics, and stakeholders to be involved in CBME planning	- MB representative(s) - ACGME Review Committee leaders - Education leaders within your specialty - Program directors - Department chairs - Academic leaders - Patients and families	- Convene a working group to explore - Identify current educational gaps in your specialty (i.e., readiness for practice; readiness for trainees) - Explore development of clear outcomes in your specialty to patients and families and expected of graduating trainees - Review the assessment practices and current time/volume requirements for certification in your specialty (Historical; Evidence from literature; Patient and public needs)	- Resistance to change within the specialty - Data collection and future processes - Unclear about the needs of patients and the public	12 mos
Before next stage: Review data regarding readiness to practice of new graduates in the specialty and share this information widely to help define the 'why' for CBME				
Developing Partner with stakeholders to create a detailed plan for the transition to CBME including consideration for how GME CBME efforts connect to certification processes	In addition to above, educational assessment experts in your specialty and from other specialties may need multiple workgroups	- Communicate current educational and practice gaps to faculty and faculty awareness that are described above - Finalize definitions of expected educational outcomes (e.g., entrustable professional activities (EPAs)) - Identify faculty development materials - Identify technological solutions for implementation - Develop or assess the assessment tools for implementation by GME programs, MB, or both - Determine how certification process might need to evolve to integrate CBME principles and assessment	- Local barriers to data collection by programs - Resource limitation (people & financial) at the local level to lead/guide CBME efforts and at the MB level to integrate CBME principles into certification processes	12-18 mos
Before next stage: Create clearly defined outcomes expected of practicing physicians in the specialty, develop a plan for pilot testing in a few sites that can be expanded to more sites as your specialty learns about what works, for whom, and in what circumstances				
Pilot Testing Begin testing the pieces (assessment tools, faculty development, technology) needed for the transition to CBME. Develop specific approaches to integrating data into certification processes	- As above - Enlist innovator and early adopter programs - Internal MB stakeholders responsible for integrating CBME data into certification processes	- Identify specific programs for piloting - Work as needed to develop a collection and assessment tools - Test and refine processes to prepare for implementation as needed/appropriate	- As above - Consider how to balance data from GME programs for certification-decision making with independent MB assessments	12-24 mos
Before next stage: Assess pilot testing to understand the multiple, complex educational and system contexts of implementation				
Implementing Begin the transition from current educational system to the new CBME system including collecting CBME data for certification processes	As above	- Communicate outcomes framework as core to training in your specialty - Disseminate tools, assessments, and faculty development materials - Continue data collection and research/quality improvement related to pilot testing - Develop a plan for how CBME data will be used (e.g., advancement, remediation, board certification)	- As above - Creating consistency across programs regardless of local resources available - Culture change needed to change approach to assessment - Learn from pilot testing to refine CBME implementation - Determine the balance between data from GME programs for certification with independent MB assessments	12-24 mos
Before next stage: Complete implementation throughout the specialty and develop a strategy for integration of CBME-related data from training into certification processes. Share outcomes widely and improve processes based on outcomes.				
Integrating & Maintaining	As above	- Adapt CBME processes as needed to continue to improve assessment and decision-making - Continue data collection and research/quality improvement related to pilot testing - Use CBME data for	As above	Ongoing



Promoting Professionalism Action Collaborative (PPAC) 2024-present

Key Components of ABMS Position Statement on Promoting Professionalism (2024)

Core Definition of Professionalism

Professionalism is a social contract based on trust, accountability, ethics, and continuous improvement in medicine.

Dynamic Nature of Professionalism

Professionalism evolves with a physician's career and changing healthcare systems, reflecting behaviors and values.

Ethical Practice and Accountability

Physicians must act with integrity, honesty, respect, and fairness, ensuring stewardship and quality improvement.

Role of Member Boards

Member Boards operationalize professionalism through certification, assessment, and supporting environments for physicians.

PROMOTING PROFESSIONALISM ACTION COLLABORATIVE 2024-PRESENT

Goal: To identify and develop resources and programs that promote professionalism competencies in medical specialties

Objectives: At the end of the event, participants will be able to:

1. Explore strategies that can be used in approaches to integrating professionalism assessment into initial and/or continuing certification.
2. Identify resources, programs, and opportunities in promoting professionalism competencies, such as self-awareness, teamwork, and communication skills.
3. Develop a work plan for integrating professionalism into initial and/or continuing certification or formulate actionable strategies to promote professionalism competencies.
4. Disseminate ideas and best practices on implementation of professionalism competency activities.

AGENDA OVERVIEW

Time	Topic
6:45 – 7:30 am	Breakfast
7:30 – 8:15 am	Overview of Meeting and Introductions • Table Discussion
8:15 – 9:30 am	Session I: Integrating Professionalism into initial and continuing certification (Part I) • Team Discussion/Work
9:30 – 9:45 am	Break
9:45 – 11:00 am	Session I: Integrating Professionalism into initial and continuing certification (Part II) • TRIZ exercise
11:00 – 11:30 am	Lunch

AGENDA OVERVIEW

Time	Topic
11:30 – 12:00 pm	Plenary: Professionalism and Patient Trust
12:00 – 1:15 pm	Session II: Resources, Programs and Opportunities in Promoting Professionalism • Team Discussion/Work
1:15 – 1:30 pm	Break
1:30 – 3:00 pm	Session III: Developing Work Plans for Professionalism Competency Assessment • Information gathering and Team Discussion/Work
3:00 – 3:25 pm	Member Board Team Discussion and Report Out
3:25 – 3:30 pm	Closing Remarks

PROFESSIONALISM OPPORTUNITIES ACROSS MEMBER BOARDS

Theme	Number of Member Boards
Clarify professionalism expectations and scope	5
Integrate professionalism into continuing certification	6
Use assessment tools and certification touchpoints	5
Strengthen communication-focused professionalism	3
Use data and feedback to guide education and early intervention	3
Explore innovation and future planning	3

PROFESSIONALISM OPPORTUNITIES ACROSS MEMBER BOARDS

- Boards are exploring ways to define professionalism more clearly and connect expectations to certification, training, and continuing certification.
- Common opportunities include adding professionalism-focused learning, assessment, and feedback into existing processes with minimal added burden.
- Several boards are testing or considering new tools, including article-based questions, communication assessments, workplace-based feedback, EPAs, oral exam criteria, and AI-supported approaches.
- There is shared interest in using data and feedback to identify professionalism themes, guide education, and support earlier intervention when concerns arise.

Professionalism Assessment from “Residency to Retirement”

Lisa Taitsman, MD, MPH
ABOS Board of Directors



American Board of Orthopaedic Surgery

Establishing Education & Performance Standards for Orthopaedic Surgeons

ABOS Professionalism

The American Board of Orthopaedic Surgery's guidelines on Professional Behavior, Respect, Compassion, and Responsibility are listed below. Professionalism is demonstrated in practice through behaviors, attitudes, and conduct that reflect responsibility and accountability for care of patients, colleagues, and our profession, including:

1. **Conduct:** Behavior and attitude that are respectful, compassionate, honest, and demonstrate integrity in interactions with patients, other professionals, healthcare systems, and payers.
2. **Privacy:** Respect for and commitment to honoring patients' rights regarding their medical and surgical care, including shared decision-making and the right to privacy.
3. **Compassionate Care:** Empathy and compassion in addressing the needs of the patient and their family, while maintaining appropriate doctor-patient relationships. This includes awareness of one's own personal implicit biases.
4. **Communication:** Honesty and integrity in communication and interaction with patients, placing patient well-being foremost when disclosing information and recommending treatment. Optimizing communication with other healthcare professionals—particularly direct physician-to-physician communication—to enhance the quality and timeliness of care.
5. **Respect:** Adherence to appropriate professional standards when critiquing care provided by previous health care professionals. Acceptance of responsibility for one's own actions. Respect for the opinions of others, and the ability to provide and accept constructive, objective criticism in the delivery of medical care and education.
6. **Social Contract:** Fulfillment of the obligations of the medical profession inherent in the social contract, including providing care for ill or injured patients regardless of personal characteristics or socioeconomic factors, and being available and responsible for time-sensitive patient care issues. The surgeon's skills should serve and meet the needs of society.
7. **Integrity:** Demonstration of integrity in interactions with other professionals in the medical, legal, and governmental community. Honesty in providing expert testimony, consulting, royalty and speaking agreements, media presentations, research, and publications.
8. **Stewardship:** Recognition of the surgeon's responsibility to manage the resources, trust, knowledge, and future of the profession. Surgeons should strive to be good stewards of limited resources and serve as mentors, educators, researchers, and volunteers within our professional and personal communities. This includes a commitment to life-long learning.
9. **Wellness:** Commitment to personal and team well-being, recognizing that caring for oneself is essential to providing the highest quality of care for the public.

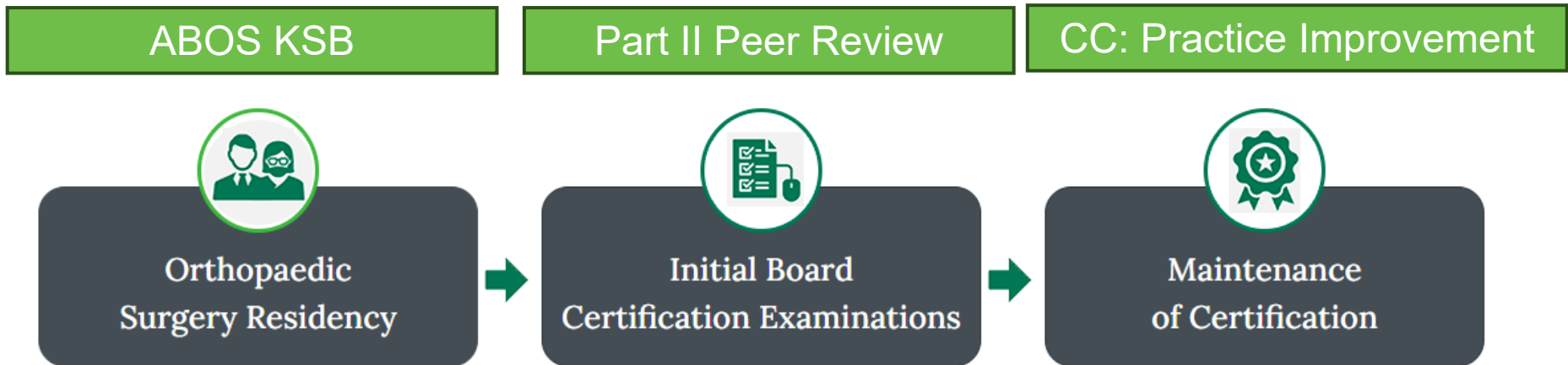
Professionalism

ABOS Residency to Retirement Roadmap

This infographic guides you through 3 stages of your orthopaedic surgery career.

The ABOS recommends that you carefully review the appropriate Rules and Procedures for Residency, Initial Board Certification, and Maintenance of Certification. The ABOS reserves the right to make changes in its rules and procedures for its examination and certification at any time and without prior notice. This infographic is a visual aide and may not appropriately reflect your pathway.

Click on a stage to see more details.



ABOS Knowledge, Skills, and Behavior Program

Assessments

KNOWLEDGE

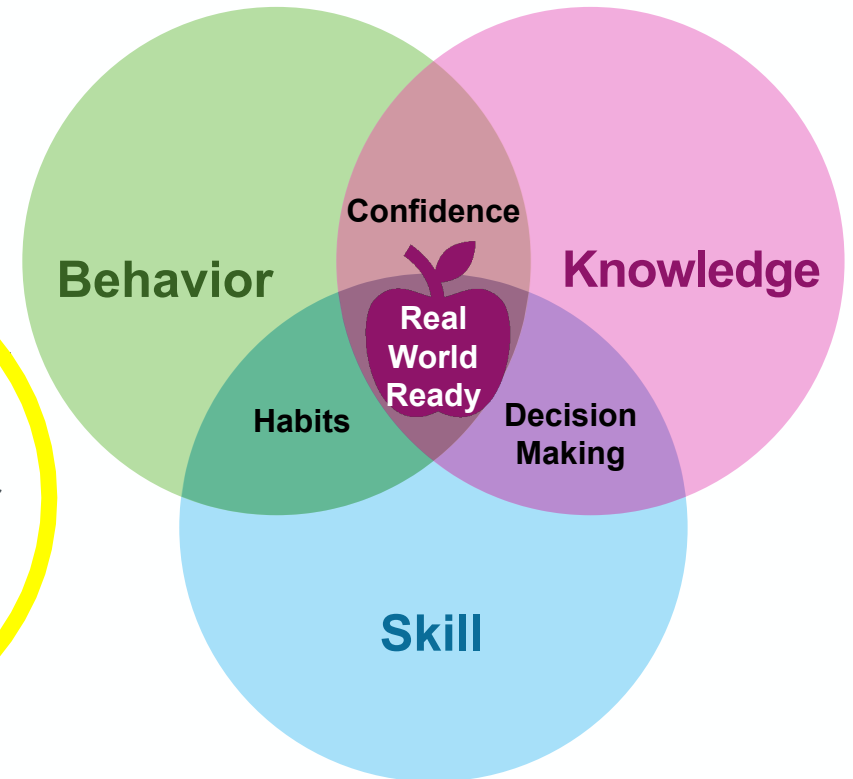
- ABOS Part I Examination Blueprint
- Defines a knowledge base – link with OITE (AAOS)
- Core Curriculum

SURGICAL SKILLS

- App Based Surgical Skills Assessment Tool
- Case Entry coordinated with ACGME

BEHAVIOR

- Professional Behavior Assessment Tool with same APP

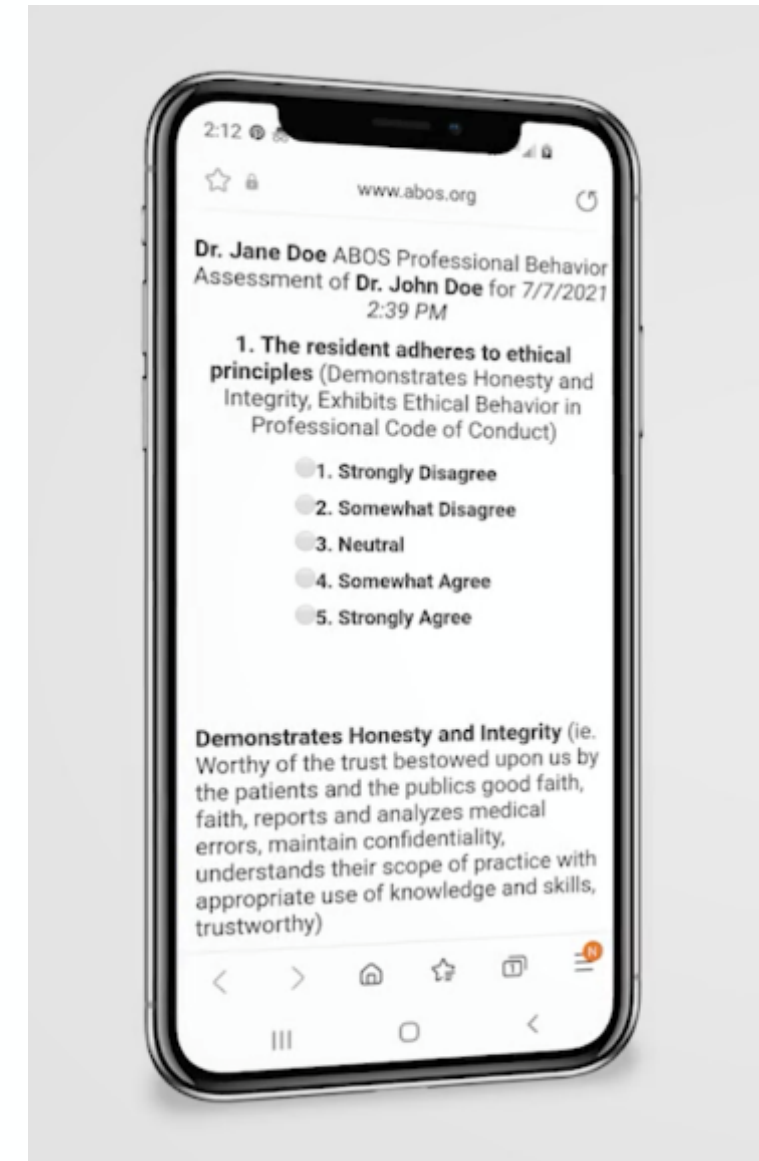


Participation Required to Sit for the ABOS Part I Examination

Professional Behavior

- The ABOS has developed a Professional Behavior tool that allows program directors and faculty members the opportunity to evaluate the professional behavior of orthopaedic residents in five domains:

1. **Ethical behavior**
2. **Communication**
3. **Interaction**
4. **Reliability**
5. **Self-assessment**



Professional Behavior Assessment

Dr. Jane Doe ABOS Professional Behavior Assessment of Dr. James Doe for 6/16/2021 10:14 AM

2. The resident effectively communicates with patients and with people who are important to those patients (Shows Compassion/Empathy, Demonstrates Communication and Listening skills, Shows Respect for Patient needs)

- ☐ 1. Strongly Disagree
- ☐ 2. Somewhat Disagree
- ☐ 3. Neutral
- ☐ 4. Somewhat Agree
- ☒ 5. Strongly Agree

Previous

Next

Shows Compassion/Empathy (ie. Collaborates with patient, enhances the relationship)

Demonstrates Communication and Listening skills (ie. Attentive, shows patience, respect patient autonomy and empower them to make informed decisions, manages communication challenges with patients and families)

Shows Respect for Patient needs (ie. Respects patients' viewpoints and considers his/her opinions when determining healthcare decisions, regards the patient as a unique individual, treats the patient in the context of his/her family and social environment, takes time to educate the patient and their family)



Professional Behavior Assessment

- There are two part of ABOS Professional Behavior Assessments
 - End of every rotation from each Faculty Member
 - 5-question survey
 - One additional, open-ended question that provides faculty the opportunity to provide comments
 - 360-Evaluation- Annual push from ABOS
 - Faculty
 - Nurses, staff, coordinators, ED providers, APPs, etc
 - Residents, faculty

***Professional Behavior* assessments go directly to Program Director/Program Coordinator and/or APD annually ***



July 2019

18 Residency Programs



THE AMERICAN ORTHOPAEDIC ASSOCIATION®
Leading the profession since 1887

AOA Critical Issues in Education

Use of the Behavior Assessment Tool in 18 Pilot Residency Programs

April D. Armstrong, MD, FAOA, Julie Agel, MA, Matthew D. Beal, MD, FAOA, Michael S. Bednar, MD, FAOA,
Michelle S. Caird, MD, FAOA, James E. Carpenter, MD, FAOA, Stuart T. Guthrie, MD, FAOA, Paul Juliano, MD, FAOA,
Matthew Karam, MD, FAOA, Dawn LaPorte, MD, FAOA, J. Lawrence Marsh, MD, FAOA, Joshua C. Patt, MD, FAOA,
Terrance D. Peabody, MD, FAOA, Karen Wu, MD, FAOA, David F. Martin, MD, FAOA,
John J. Harrast, MS, and Ann E. Van Heest, MD, FAOA

Background: The purpose of this study was to determine the feasibility and evaluate the effectiveness of the American Board of Orthopaedic Surgery Behavior Tool (ABOSBT) for measuring professionalism.

Methods: Through collaboration between the American Board of Orthopaedic Surgery and American Orthopaedic Association's Council of Residency Directors, 18 residency programs piloted the use of the ABOSBT. Residents requested assessments from faculty at the end of their clinical rotations, and a 360° request was performed near the end of the academic year. Program Directors (PDs) rated individual resident professionalism (based on historical observation) at the outset of the study, for comparison to the ABOSBT results.

Results: Nine thousand eight hundred ninety-two evaluations were completed using the ABOSBT for 449 different residents by 1,012 evaluators. 97.6% of all evaluations were scored level 4 or 5 (high levels of professional behavior) across all of the 5 domains. In total, 2.4% of all evaluations scored level 3 or below reflecting poorer performance. Of 431 residents, the ABOSBT identified 26 of 32 residents who were low performers (2 or more < level 3 scores in a domain) and who also scored "below expectations" by the PD at the start of the pilot project (81% sensitivity and 57% specificity),

continued

5 domain tool
web-based ABOS
Behavior Tool

End of rotation
and 360

97.6% good
behavior

360 degree
evaluation
identified more
low performers

Requirement

Annual Requirement starting academic year 2025-2026:

Completed assessments:

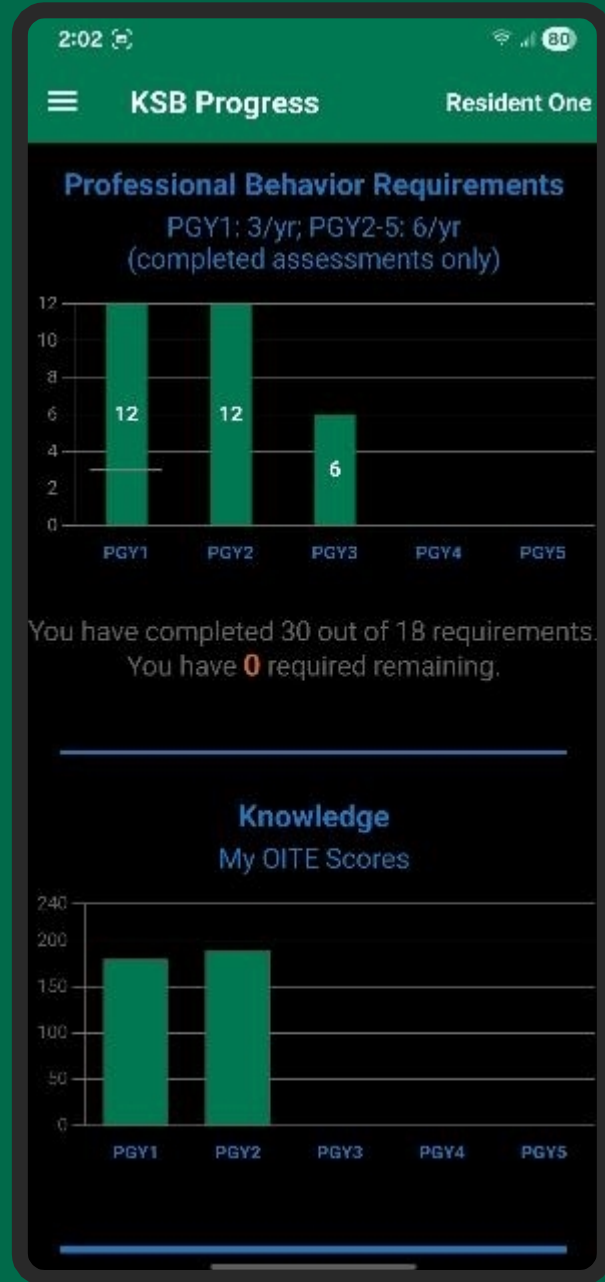
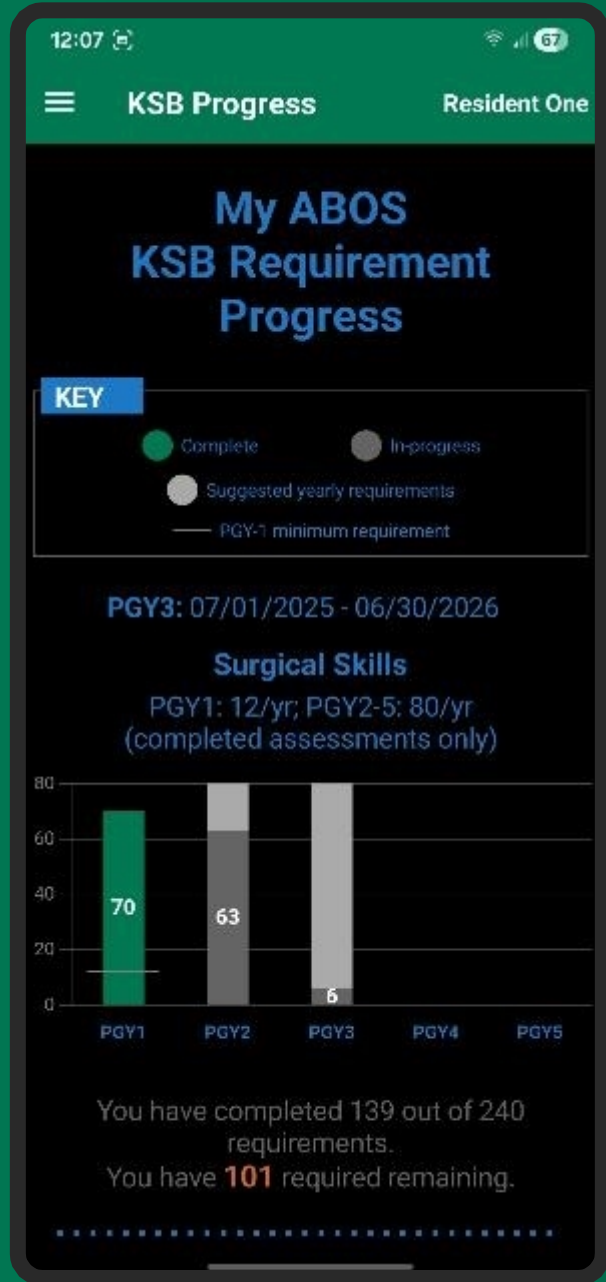
- **80 Surgical Skills**
- **6 Professional Behavior**
- **1 Orthopaedic In-Training Examination**

Effective July 1, 2025, for Residents who wish to take the ABOS Part I Examination in July 2026 and beyond

- **First year as a requirement:**

100% participation





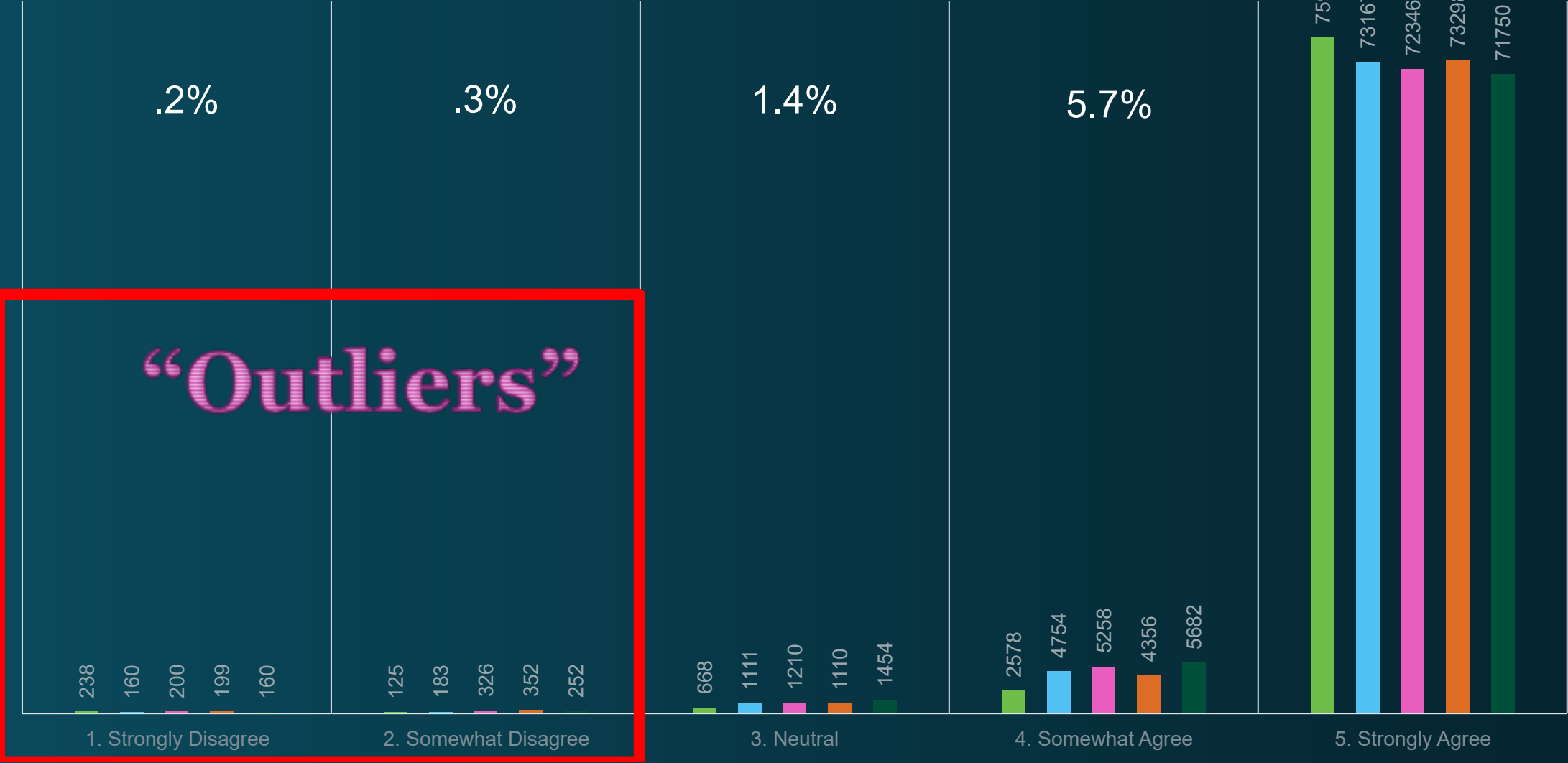
ABOS KSB Requirement

- Participation requirement must be met to allow resident to sit for ABOS Part I examination
- Program Director has ultimate discretion on resident competence, provided participation requirement and other ABOS residency requirements are met
- Programs and Residents can track resident progress



DISTRIBUTION OF PROFESSIONAL BEHAVIOR SCORES 2025-2026

■ Ethical Behavior ■ Communication ■ Interaction ■ Reliability ■ Self-Assess



Professional Behavior 2025-2026 Only

Distribution of ~80,000+ ABOSBT assessments across each of five domains.

Red Flag Assessments=

Ethics= .46%

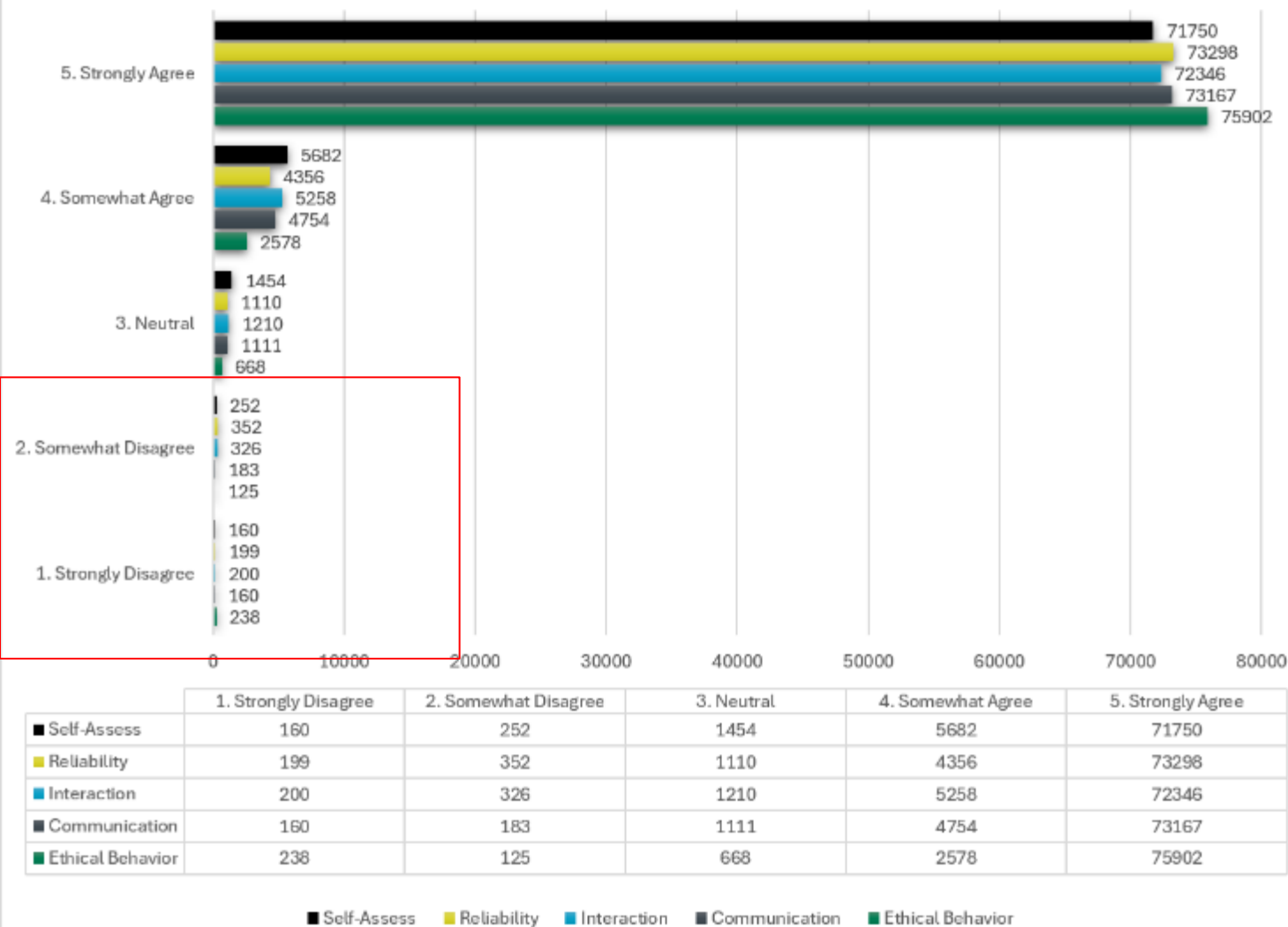
Communication=.44%

Interaction=.67%

Reliability=.70%

Self-Assessment=.52%

Professional Behavior Assessment by Ratings and Domain for 2025-2026



Resident Examples

- Resident 1:
 - Great job on rotation. Reliable, thoughtful and organized. Did great job managing team. Excellent communicator. Prepared for cases. Very good interactions with patients and staff and entire team. Hard worker. Thank you! (Attending)



Resident Examples

— Resident 2:

- Unreliable, often late or unprepared for patient rounds. Frequently demonstrates lack of understanding on patient care and plans. Poor communication skills with OR staff and nursing staff (Resident)
- Frequently disregarded hospital policy on sterility, even after having it brought to attention. Was not receptive to any critique and was often on personal phone following the procedure. Quite disrespectful to members of the OR team. (OR Nurse)
- Continues to mismanage patients and has become untrustworthy regarding basic points of knowledge for year in training. (Attending)

Resident Examples

- Resident 3:
 - Try to keep sexual comments to a minimum in the OR, especially when around providers that have not worked with you before. Recommend being kinder with feedback as to not come off as arrogant or rude. If you start a conversation with "no offense" it usually is offensive and you may want to rethink the rest of the sentence. overall, you were good about being professional in front of the patients or during morning sign-out (Inpatient Staff)



Resident Examples

– Resident 4:

- Not receptive to constructive criticism. Often talks down to other members of the care team. Poor communication skills and documentation skills. Does not hold self accountable. (Inpatient Nurse)
- Fails to provide the correct medical care and utilize the appropriate consult services when caring for complex medical patients. Doesn't follow up on nursing needs/ patient issues throughout the shift. Often responds to pages in an arrogant and condescending manner. (Inpatient Nurse)
- OR staff, nursing, other residents have had negative interactions which I think are in part communication style, not person. People may cite instances of passing off work, I have personal instances of doing favors and taking on work. Well-read, technically solid from what I have heard. Just needs to listen to others and not assume a confrontational stance to feedback (Resident)
- Needs to work on interactions with support staff in operating rooms (Attending)

Resident Examples

— Resident 5:

- Extremely concerning behavior on many levels! Takes guidance very poorly. Defensive in responses. Lazy; constantly delegates work to other residents. Very little self-awareness, and even more importantly, self-correcting behavior. Lack compassion for patients. Overall, mix of multiple traits that, in my opinion, makes very dangerous to patients...(Attending)

STATE

Surgeon who left patient charged with child rape

THE ASSOCIATED PRESS

Published 12:00 a.m. ET Sept. 11, 2002 | Updated 4:04 p.m. ET Jan. 12, 2011

[View Comments](#)



CAMBRIDGE -- The Boston surgeon who left a patient on the operating table for 35 minutes while he went to cash a check was arrested yesterday and charged with raping a 15-year-old boy.

THE SPINAL TAP

Florida Spine Surgeon and Device Company Owner Charged in Kickback Scheme

NEWS

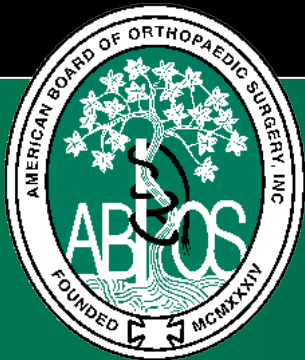
Greenwich MD gets 5 years in prison for insider trading





Peer Review Program

Initial Certification
Continuing Certification

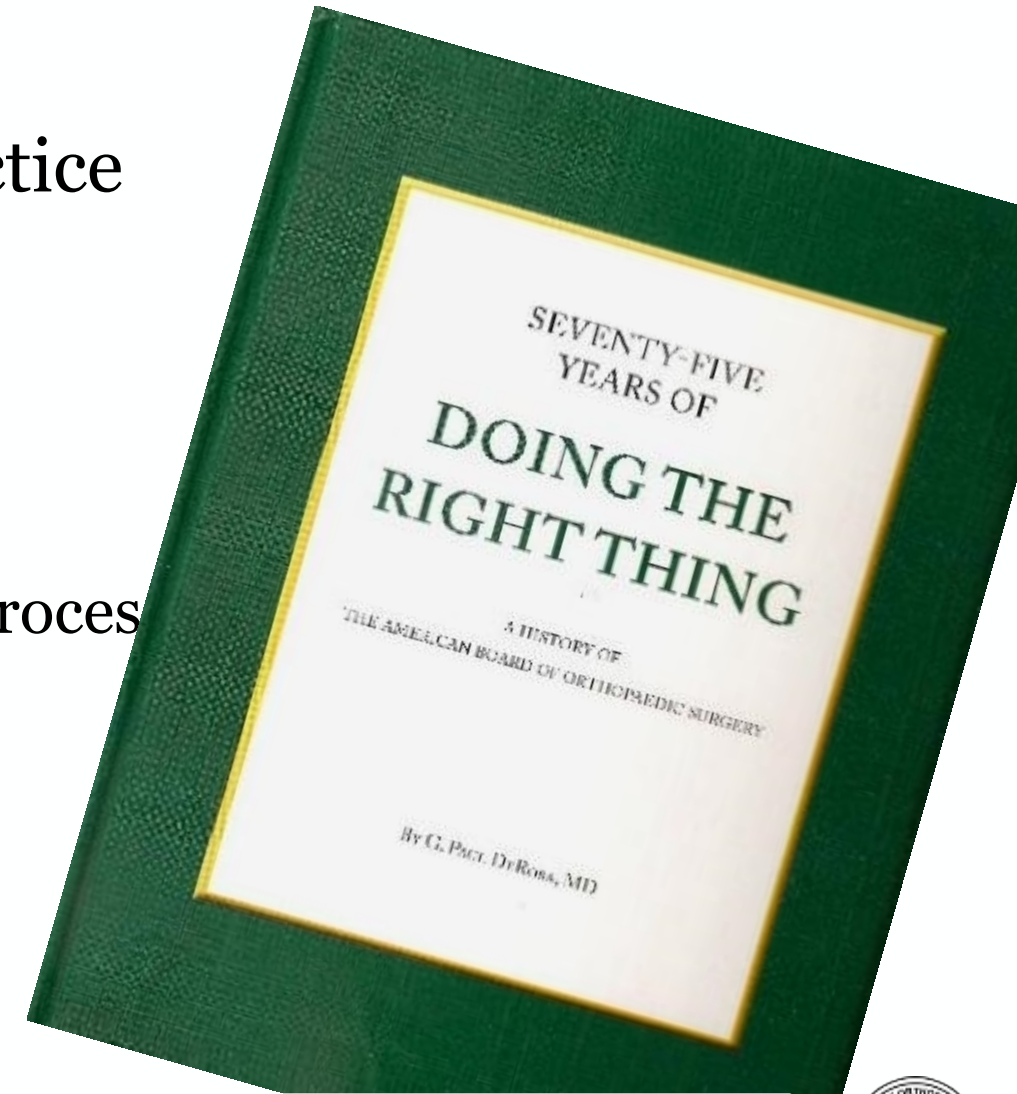


American Board of Orthopaedic Surgery

Establishing Education & Performance Standards for Orthopaedic Surgeons

ABOS Peer Review

- Professionalism and Performance in Practice
 - Initial certification
 - Recertification
- Allows in-depth evaluation
 - Not available elsewhere in the certification process
 - Insight into practice activities
 - Fair, anonymous process
- Separates us from other Boards



“Do the Right Thing...even when it’s hard!” *-Rick Wright*



CONFIDENTIAL REFERENCE

The AMERICAN BOARD of ORTHOPAEDIC SURGERY
(INCORPORATED 1934)

PRESIDENT
MELVIN S. HENDERSON
ROCHESTER, MINN.
VICE PRESIDENT
EDWIN W. RYERSON
SECRETARY-TREASURER
FREMONT A. CHANDLER
180 NORTH MICHIGAN AVE.
CHICAGO, ILL.

GEORGE E. BENNETT
WILLIS C. CAMPBELL
SAMUEL KLEINBERG
PHILIP LEWIN
JOHN C. WILSON
PHILIP D. WILSON

TO

Dr. John S. Horsley

617 W. Grace Street,

Richmond, Va.

CHICAGO, ILL., 9-24-37 19

DEAR DOCTOR:

Will you kindly give this Board information regarding the professional standing and character of the applicant for certificate, noted below.

This request is unknown to the candidate and all data you give will be held in strict confidence.

Your cooperation will be appreciated by the members of the Board.

Kindly return this form to the Secretary at your earliest convenience.

Very sincerely,

FREMONT A. CHANDLER, Secretary

Name of Applicant

Dr. [REDACTED]

Address

[REDACTED] Va.

1. Training: Excellent.

2. Professional Standing: Fair.

3. Comments relative to the applicant's ability in Orthopaedic Surgery.

• [REDACTED] is a very capable man, but
I do not regard him as a very high character.

(Signature)

(Use reverse side for additional data)

REFERENCE SUPPLIED BY CANDIDATE

The AMERICAN BOARD of ORTHOPAEDIC SURGERY
(INCORPORATED 1934)

PRESIDENT
MELVIN S. HENDERSON
ROCHESTER, MINN.
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GEORGE E. BENNETT
WILLIS C. CAMPBELL
SAMUEL KLEINBERG
PHILIP LEWIN
JOHN C. WILSON
PHILIP D. WILSON

TO

Dr. George E. Bennett

4 E. Madison St.,

Baltimore, Md.

CHICAGO, ILL., 9-11-37 19

DEAR DOCTOR:

Will you kindly give this Board confidential information regarding the professional standing and character of the applicant for certificate, noted below. He has given you as a reference. Your cooperation will be appreciated by the members of the Board.

Kindly return this form to the Secretary at your earliest convenience.

Very sincerely,

FREMONT A. CHANDLER, Secretary

Name of Applicant

Dr. [REDACTED]

Address

[REDACTED] Virginia

1. Training: I know nothing of [REDACTED] training.

2. Professional Standing: I know of no particular unethical thing
that he has done. However, he is always in trouble
with the Board of the orthopaedic men in the State of
Virginia.

3. Comments relative to the applicant's ability in Orthopaedic Surgery.

I think he is a first class orthopedic surgeon.

(Signature)

(Use reverse side for additional data)

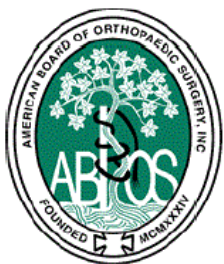


ABOS Peer Review

HOW IS IT DONE TODAY?

- Electronic Application (Professional Standing Update)
 - ***Names/email addresses requested***
 - Partners
 - ABOS Diplomates (5)
 - For each hospital/surgicenter/institution
 - *Chief of Staff*
 - *Chief of Surgery*
 - *Chief of Orthopaedics*
 - *Chiefs of Anesthesia, Radiology, Emergency Medicine*
 - *OR Nursing Supervisor/Ortho Nursing Supervisor*
 - ***Zip Code Evaluation***
- Minimums: 7 completed/5 orthopaedic evaluations
 - Rural Waiver/Executive Staff Review





American Board of Orthopaedic Surgery

Peer Review

The American Board of Orthopaedic Surgery (ABOS) appreciates Peer Review by orthopaedic surgeons, other physicians, and non-physician professionals. You need not be an orthopaedic surgeon or be Board Certified by the ABOS to complete this Peer Review Survey.

Please complete all of the information before submitting.

Peer Review of:

[Redacted Name]

1. Which best represents your relationship to the applicant? (Select one)

- ☐ Orthopaedic Colleague
- ☐ Current Practice Partner
- ☐ Former Practice Partner
- ☐ Chief of Orthopaedics
- ☐ Chief of Surgery
- ☐ Chief of Anesthesiology
- ☐ Chief of Staff
- ☐ Other (please state):

2. Which best represents your familiarity with the practice of [Redacted Name]

- ☐ Not familiar
- ☐ Slightly familiar
- ☐ Moderately familiar

Please evaluate [Redacted Name] in the following areas:

PATIENT CARE				
Unsatisfactory	Marginal	Satisfactory	Excellent	Unable to evaluate
1	2	3	4	UA
Poor clinical skills; poor judgement; disregard of patient preferences; inappropriate/ineffective care		Excellent clinical skills; sound judgement; respect of patient preferences; appropriate/effective care		

MANAGEMENT OF MULTIPLE COMPLEX PROBLEMS				
Unsatisfactory	Marginal	Satisfactory	Excellent	Unable to evaluate
1	2	3	4	UA
Very limited ability to manage patients with multiple complex medical and surgical problems		Excellent ability to manage patients with multiple complex medical and surgical problems.		

SURGICAL SKILLS				
Unsatisfactory	Marginal	Satisfactory	Excellent	Unable to evaluate
1	2	3	4	UA
Unsafe, inadequately prepared, indecisive, lacking knowledge, not surgically proficient, limited medical knowledge		Safe, prepared, decisive, knowledgeable, surgically proficient, exceptional medical knowledge		

COMMUNICATION AND INTERPERSONAL SKILLS				
Unsatisfactory	Marginal	Satisfactory	Excellent	Unable to evaluate
1	2	3	4	UA
Poor relationships/communications with patients/colleagues; not respectful or compassionate; avoid educating and counseling patients; accepts counsel poorly		Excellent relationships/communications with patients/colleagues; respectful and compassionate; educates and counsels patients; accepts counsel well		

PROFESSIONAL BEHAVIOR				
Unsatisfactory	Marginal	Satisfactory	Excellent	Unable to evaluate
1	2	3	4	UA
Not professional/proper, avoids responsibility for errors; lack of respect for patients and colleagues		Professional/proper, accepts responsibility for errors; respects patients and colleagues		

INTEGRITY AND ETHICAL VALUES				
Unsatisfactory	Marginal	Satisfactory	Excellent	Unable to evaluate
1	2	3	4	UA
No integrity or ethical values, low character; inadequate commitment to honesty		High integrity and ethical values, high character; exceptional commitment to honest		

To your understanding, does [Redacted Name] practice safe, ethical, and effective orthopaedic surgery and meet the highest standards of practice and conduct?

☐ Yes, I feel [Redacted Name] meets the highest standards of practice and conduct.

☐ No, I do not feel [Redacted Name] meets the highest standards of practice and conduct.

☐ I feel [Redacted Name] meets the standards of practice and conduct with the following reservations:

If no, or with reservations, please explain and list anyone the board should contact. Please limit entry to 255 characters:

Electronic Signature

☐ By checking this box, I attest that the responses recorded above have been provided by [Redacted Name] and that checking this box constitutes [Redacted Name] electronic signature.

PHONE NUMBER

Finalize Form

2023 Recert App ID: [REDACTED]

Total requested: 28

Total responded: 21

Total sufficiently complete: 21

Orthopaedic responded: 17

Orthopaedic sufficiently complete: 17

Name Submit Date	Fam	Relationship	City, State Phone	S	O	V	Pt Care	Mgmt Cplx	Surg Skill	Comm Skills	Behavior	Integ
ORTHO: [REDACTED] 10/13/2022	V	Ortho Colleague	[REDACTED]	Y	1	1	4	4	4	4	4	4
ORTHO: [REDACTED] 10/12/2022	V	Current Partner	[REDACTED]	Y	1	1	4	4	4	4	4	4
ORTHO: [REDACTED] 01/16/2023	M	Fmr Partner	[REDACTED]	Y	1	1	4	4	4	4	4	4
ORTHO: [REDACTED] 10/12/2022	SI	Ortho Colleague	[REDACTED]	Y	1	1	4	4	4	4	4	4
ORTHO: [REDACTED] 10/12/2022	V	Ortho Colleague	[REDACTED]	Y	1	1	4	4	4	4	4	4
ORTHO: [REDACTED] 10/12/2022	V	Current Partner	[REDACTED]	Y	1	1	4	4	4	4	4	4
ORTHO: [REDACTED] 11/16/2022	V	Current Partner	[REDACTED]	Y	1	1	4	4	4	4	4	4
ZIP: ORTHO: [REDACTED] 10/17/2022	M	Current Partner	[REDACTED]	Y	1	1	4	4	4	4	4	4
ZIP: ORTHO: [REDACTED] 10/12/2022	M	Current Partner	[REDACTED]	Y	1	1	4	4	4	3	4	4
ORTHO: [REDACTED] 10/13/2022	V	Ortho Colleague	[REDACTED]	Y	1	1	4	4	4	4	4	4
ORTHO: [REDACTED] 10/12/2022	V	Ortho Colleague	[REDACTED]	Y	1	1	4	4	4	4	4	4
ORTHO: [REDACTED] 10/14/2022	M	Ortho Colleague	[REDACTED]	Y	1	1	4	4	4	4	4	4



2023 Part II App ID: [REDACTED]

Total requested: 60

Total responded: 19

Total sufficiently complete: 19

Orthopaedic responded: 17

Orthopaedic sufficiently complete: 17

Name Submit Date	Fam	Relationship	City, State Phone	S	O	V	Pt Care	Mgmt Cplx	Surg Skill	Comm Skills	Behavior	Integ
ORTHO: [REDACTED] 10/31/2022	V	Current Partner	[REDACTED]	N	1	1	②	①	②	②	②	②
ORTHO: [REDACTED] 10/31/2022	V	Current Partner	[REDACTED]	N	1	1	②	②	②	②	②	①
ORTHO: [REDACTED] 11/11/2022	V	Fmr Partner	[REDACTED]	N	1	1	②	②	3	②	②	②
ORTHO: [REDACTED] 02/27/2023	V	Fmr Partner	[REDACTED]	N	1	1	②	②	3	②	②	①
ORTHO: [REDACTED] 11/14/2022	V	Current Partner	[REDACTED]	R	1	1	3	②	②	②	②	②
ORTHO: [REDACTED] 11/07/2022	V	Ortho Colleague	[REDACTED]	N	1	1	②	3	3	②	②	②
ORTHO: [REDACTED] 12/09/2022	M	Current Partner	[REDACTED]	N	1	1	3	②	U	3	②	3
ORTHO: [REDACTED] 01/06/2023	V	Current Partner	[REDACTED]	R	1	1	3	3	3	4	②	②
ORTHO: [REDACTED] 11/01/2022	M	Fmr Partner	[REDACTED]	N	1	1	②	U	3	②	3	3
ORTHO: [REDACTED] 11/01/2022	M	Current Partner	[REDACTED]	R	1	1	4	4	4	4	②	4
ORTHO: [REDACTED] 11/03/2022	SI	Ortho Colleague	[REDACTED]	Y	1	1	4	4	4	4	4	4
ORTHO: [REDACTED] 12/01/2022	V	Ortho Colleague	[REDACTED]	Y	1	1	4	4	4	4	4	4



Name	Comment
ZIP:ORTHO: [REDACTED]	Provider routinely does not follow through with patient care and complications, communication regarding patient care is poor and self awareness is poor. Primary concern is poor follow up and care for patients with complications.
ZIP:ORTHO: [REDACTED]	Unwilling to identify or take responsibility for patient complications. Contact [REDACTED] for further details.
ZIP:ORTHO: [REDACTED]	I have known [REDACTED] for more than a decade, and without reservation [REDACTED] is the worst partner I've ever encountered. [REDACTED] abandons patients, ignores complications, is unaccountable to [REDACTED] colleagues, refuses coaching, lacks empathy, and is dangerous.
ZIP:ORTHO: [REDACTED]	Patient satisfaction needs more attention
ORTHO: [REDACTED]	I have limited familiarity with [REDACTED] and [REDACTED] practice but am unaware of any concerns that would cause me to recommend against board recertification.
ZIP:ORTHO: [REDACTED]	Could work on improving interactions with colleagues , and spend more time explaining things to patients
[REDACTED]	[REDACTED] can perform a hip or knee replacement and that is it. [REDACTED] has no empathy. [REDACTED] does not take care of [REDACTED] complications. [REDACTED] will not aspirate an infected knee as [REDACTED] says [REDACTED] does not feel comfortable doing it. [REDACTED] doesn't answer calls when [REDACTED] is on-call.
[REDACTED]	Unfortunately, there are examples of [REDACTED] practice choices that do not align with what [REDACTED] partners would do nor standards expected by the ABOS. [REDACTED] working relationship within the total joint subgroup is subpar without effort from [REDACTED] to improve it.

Fam = Familiarity with applicant's practice: N = None, Sl = Slight, M = Moderate, V = Very

O = Orthopaedic

V = Valid = 3 or more of 6 ratings complete = sufficiently complete



Further Review

(1's/2's)

- Personal Calls
- Candid, anonymous feedback
- Elaborate on ratings
 - Probing questions
 - Experience/Patience/Listening
- Should candidate sit for exam?
- What type of exam?
- Improving? / Regressing?
- *Executive Report generated*

ABOS Credentials

Spring 2023

CANDIDATE/DIPLOMATE: _____

Reference Name: _____

Reference Name: _____

Reference Name: _____



ABOS Credentials Committee

Action Options:

- *Allow to Sit*
- *Mandatory Oral Examination*
- *Defer*
- *Deny (1 to 3 years)*
- *Revoke*
- *Site Visit*
- **Appeal Process**

Results:

- **ABOS Part II Oral Examination**
 - **81/868** ➡ **27 to Committee**
 - *Action on ~15-20*
- **ABOS Recertification Examinations**
 - **257/2366** ➡ **51 to Committee**
 - *Action on ~25-30*

*40 to 50 physicians
identified with significant
issues each year*



Peer Review Results

ABOS Diplomate Issues

- “Already done with ‘360 Review’ in my hospital”
 - **Certification** versus **Privileging**
- “Allowing me to be judged by my competition”
 - Site Visit option
- “No one knows my practice”
 - **Often, they do**

ABOS Issues

- Anonymity concerns
 - Careful communication



Bottom Line on ABOS Assessments

Do these processes measure Professionalism?

- ***Yes***
- KEYS: ***Anonymity*** and ***Professionalism***

Do these processes improve Professionalism?

- ***Maybe***
- We have seen improvement

Can these processes foster Professionalism?

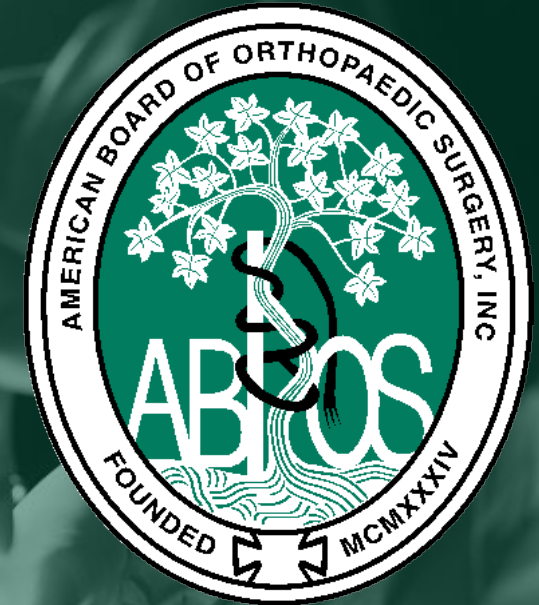
- ***Hope so...***
- Professional Behavior can be learned

“Who are you are when nobody’s looking?”



Thank you

Lisa Taitsman, MD, MPH
ltaitsman@bsd.uchicago.edu
919-929-7103



ABMS CONFERENCE 2026

Assessing professional behaviors where the work happens

An American Board of Pediatrics (ABP) funded
workplace-based assessment (WBA) platform

Ariel Winn, MD

Boston Children's Hospital / Harvard Medical School

The goal: practice ready

**“The ability to provide safe and effective care
without assigned supervision.”**

Set at the highest level of the supervision scale

**The required outcome
of training**

**Assessed where the
work happens**

**Consequential for
certification in 2028**

Winn et al., Pediatrics 2026

The story I'll tell

1

**Professional behaviors are essential
for care to be safe and effective.**

2

**Workplace-based assessment can be
leveraged to assess professional behaviors.**

Where those behaviors become visible: real encounters, in real time

What we mean by professionalism here

Would training move it?

YES

Competency

Communication · collaboration
help-seeking · reliability · honesty

→ Response: teaching

THIS IS WHAT WE ASSESS

NO

Conduct

A line crossed

→ **Adjudicated,
not coached**

Not this instrument

NO

State needing care

Impairing now, treatable

→ **Treated,
not disciplined**

Not this instrument

The instrument

One to three minutes, on a phone, in the flow of care.

EPA-based forms

Supervision rating
+ narrative comment

Narrative-only forms

Targeted at professional
behaviors, by name

Knowing Limits and Seeking Help

What best describes this assessment?

- ☐ Based on a single encounter or day
- ☐ Based on multiple observations
- ☐ End of rotation compiled feedback

Knowing one's individual limits of knowledge/ability and seeking help from others when needed are critical skills for all physicians. Provide examples of how this resident has done this well and how they can further develop in this area:

Comments must not include the patient's name or any other patient identifiers

Free text....

How the forms were chosen

Developed together, with overlapping people — not bolted on afterward.



**The KSAs were defined with assessment in mind —
built to be observable in real encounters, from the start.**

**Essential to
entrustment**

Cuts across EPAs

Observable

Developable

Each cross-cutting KSA became a form

CROSS-CUTTING KSA — ESSENTIAL TO ALL 12 EPAS

NARRATIVE-ONLY FORM IN THE APP

Recognizing Limits and Seeking Help

→ **Knowing Limits and Seeking Help**

Maintaining Professionalism

→ **Professional Behaviors and Responsibilities**

Providing Patient- and Family-Centered Care

→ **Communicating With Patients & Families /
Providing Family-Centered Care**

Communicating Clearly and Respectfully

→ **Collaborating With Others**

Promoting Health Equity

→ **Promoting Health Equity**

Committing to Lifelong Learning

→ **Commitment to Growth and Learning**

Providing High-Value Care

→ **High-Value Care**

Why narrative comments?

The richest signal in workplace-based assessment is written in words, not numbers.

**Comments alone can
reliably
rank residents**

even from the first few assessments

Ginsburg 2017

**Early narrative carries
warnings
that predict later struggle**

themes visible in the first 6 months

Kelleher & Kinnear 2021

**The performance signal
lives in the words**

scores get the reputation;
comments carry the information

Ginsburg 2013

79%

of residents: narrative helps them
understand WHY

75%

of residents: narrative helps them
know HOW to improve

81%

of faculty: the app helps them give
actionable comments

Ginsburg, van der Vleuten & Eva, Acad Med 2017 · Kelleher, Kinnear et al., Perspect Med Educ 2021 · Ginsburg, Eva & Regehr, Acad Med 2013 · WBA platform end-user survey

Promoting Health Equity

Two verbatim comments, from the same form.

CONSTRUCTIVE

“When using the interpreter, remember to use short sentences to make sure communication is not being lost.”

EXEMPLARY

“[The resident] identified a patient with substantive housing-related issues ... She did not stop at putting resources in EPIC, but also completed a letter of support for the family, informed them how to report concerns via 311, and engaged with our Social Worker to follow up with the mother and their housing complex ...”

Collaborating With Others

Two verbatim comments, from the same form.

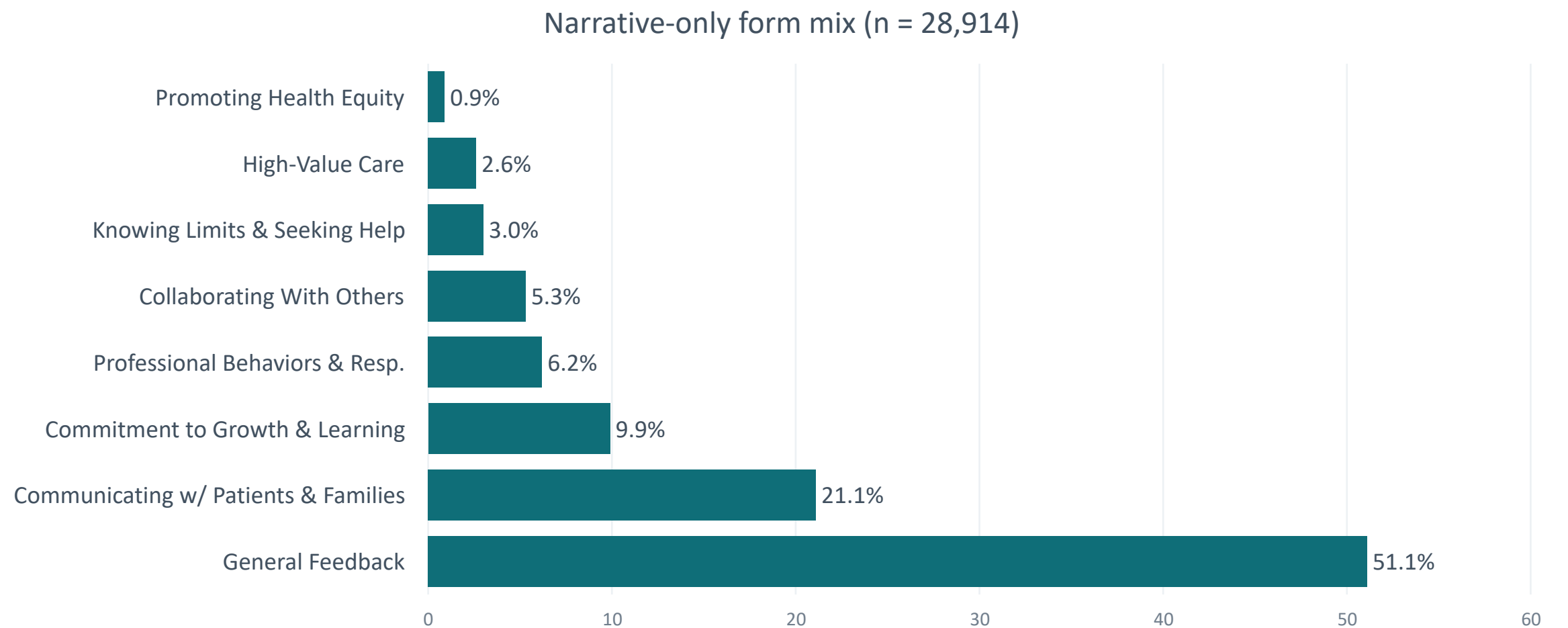
CONSTRUCTIVE

“Next steps will be to continue to work on inclusive, closed loop communication with ancillary staff (nurses, RT, etc) ... to ensure all information/points of view are taken into account and the care plans ... understood and accepted by all member of the team.”

EXEMPLARY

“While working with [the resident] ... I noted that he did a great job coordinating the management of his teams’ patients. He had a great rapport with nurses and consulting teams and was able to discuss and work through multiple complex cases with his residents and social work.”

Volume, and what actually gets used



Most comments reinforce. Fewer correct.

Our impression — not yet formally analyzed.

THE GOOD

Reinforcing feedback promotes professionalism

Not just documentation — naming these behaviors likely reinforces them, and signals to residents and faculty that they matter.

THE CONCERN

Growth edges may go unwritten

Constructive signal is sparse exactly where committees need it most — and silence is hard to interpret.

Why is this resident stalling?

Not progressing — still needing significant supervision across many EPAs.

ONE DIAGNOSIS

?

Communication

Plans are right — but supervisors re-explain them to families.

→ Observed family conversations, coaching

ANOTHER DIAGNOSIS

?

Help-seeking

Knows the medicine — but doesn't escalate, doesn't close the loop.

→ Graduated autonomy, escalation triggers

A THIRD DIAGNOSIS

?

Organization + prioritization

Tasks pile up — notes late, follow-ups dropped.

→ Workflow coaching, task management

The third isn't traditionally called "professionalism." In some ways, it is.

The ratings look identical. The narrative tells them apart.

Communication, below and above the bar

ABOVE THE BAR — STILL GROWING

Working to be more succinct in communication with patients and families.
Completing notes in a more timely fashion.

PRACTICE READY — safe and effective care without assigned supervision

BELOW THE BAR

A supervisor re-explains the plan for the family to understand it.
Documentation that lacks accuracy.
Care is not yet safe and effective.

Comments drive improvement on both sides of the bar.

Why knowing limits and seeking help?

**“The foundation of prospective
entrustment decision making.”**

What committees lean on most when deciding how much supervision a trainee will need

But committees often lack data on it — and default to advancing trainees unless red flags appear.

The studies called for exactly this data. So we built a form for it.

Schumacher, Michelson, Winn, Turner, Elshoff & Kinnear, Med Educ 2022 · Schumacher, Martini, Michelson, Turner, Winn & Kinnear, Med Educ 2025

Knowing Limits and Seeking Help

Two verbatim comments, from the same form.

CONSTRUCTIVE

"It seemed like [the resident] felt internal pressure to know what to do ... when it would have been appropriate and likely preferable to have asked for more support. It's important to ask for help ... — that's part of how you learn and reflects maturity that you understand where your gaps are!"

EXEMPLARY

"The resident did an excellent job ... During times of ambiguity and uncertainty, he appropriately reached out with a specific clinical question and well thought out clinical reasoning. I fully trusted the resident to be autonomous within his comfort level and to seek help when needed."

The second comment is an entrustment decision, in plain words.

And the EPA forms carry it too

One comment, from one EPA encounter — broken down.

FROM AN EPA FORM — CARE OF THE NEWBORN PRIOR TO DISCHARGE

*“[The resident] was **very proactive in alerting the attending quickly** when the mean arterial pressures was low on a newborn. She was able to **identify the acuity of the situation** and also provide management plans ... **very eager to learn ... asks appropriate questions** ... went through the differential diagnosis in a **very succinct yet thorough and effective way** ... I was able to **discern her clinical reasoning.**”*

KNOWING LIMITS & SEEKING HELP

Proactive, early escalation — discerning limits, seeking help when the patient is sick.

COMMITMENT TO GROWTH & LEARNING

Eager, appropriate questions —
PBLI-2: reflective practice & commitment to personal growth.

TOWARD ENTRUSTMENT

Clinical reasoning, demonstrated aloud — the evidence entrustment decisions are built on.

And the EPA forms carry it too

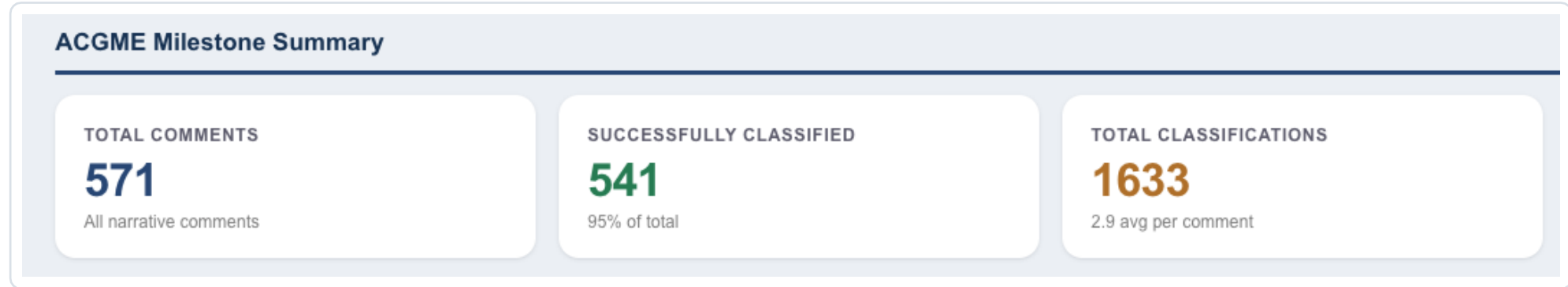
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The form maps to one EPA. The narrative is also evidence for another — EPA 7, recognizing the severely ill patient.

And it happens at scale



571 narrative comments → 1,633 milestone classifications — 2.9 per comment

**The professionalism evidence is already in there —
sortable by milestone, ready for the CCC.**

Into the entrustment decision

A committee entrusting an EPA is already making a professionalism judgment. Now it has evidence.

Documented, dated behavior — many assessors, many settings —
instead of hallway reputation.

From the frontline to the certification decision — 2028.

Questions, Comments, Discussion